

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Varivax and Varilrix for vaccination against Chickenpox

Summary of NICE and NICE CKS Guidelines for Chickenpox (Varicella) Vaccination

Vaccine Overview

- The varicella (chickenpox) vaccine is a **live attenuated vaccine**.
- Two licensed brands in the UK: **VARIVAX®** and **VARILRIX®**.
- Typically administered **subcutaneously**, but **intramuscular** administration is also acceptable if necessary.

Indications for Vaccination

The varicella vaccine is recommended for:

1. **Susceptible healthcare workers** and those in contact with immunocompromised individuals (to prevent transmission).
2. **Susceptible close household contacts** of immunocompromised patients.
3. **Non-immune adults and adolescents** (13 years and older) with no history of chickenpox, especially women of childbearing age (with appropriate contraceptive advice).
4. **Susceptible children (over 12 months)** who are at high risk of severe chickenpox or pose a risk to vulnerable contacts.
5. **Outbreak control** in some institutional settings (e.g. schools, nurseries, hospitals).

Note: "Susceptible" = no reliable history of chickenpox or no evidence of immunity.

Dosing Schedule

- **Two doses** are required for full protection:
 - **First dose** at any time from 12 months of age.
 - **Second dose** given at least **4 to 8 weeks later**.
- If given as part of MMRV combination vaccine, different guidance may apply for under-2s (due to febrile seizure risk).

Contraindications

Do **not** administer the vaccine to:

- Individuals with a history of severe allergic reaction to a previous varicella vaccine dose or any component.
- Immunosuppressed individuals (unless under specialist advice).
- Pregnant women (and advise avoiding pregnancy for **at least 1 month** after vaccination).
- People with untreated active tuberculosis or current febrile illness requiring postponement.

Precautions

- Postpone in acute illness with fever.
- Avoid salicylates (e.g. aspirin) for 6 weeks after vaccination due to Reye's syndrome risk.
- Consider prior immunity testing in adults if uncertain.

Vaccine Effectiveness

- Two doses are about **90–95% effective** at preventing chickenpox.
- If breakthrough infection occurs, it is usually mild.

Adverse Effects

- Common: Local site reactions, mild fever, mild rash.
- Rare: Febrile seizures (especially with MMRV in <2 years), varicella-like rash, mild systemic symptoms.

Documentation and Reporting

- Record batch number, site of injection, and date.
- Report any adverse reactions via **Yellow Card Scheme**.

Patient Group Direction

for the supply/administration of

Varivax

for vaccination against

Chickenpox

Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	• Pharmacist registered and practicing with the GPhC • Pharmacy technician registered and practicing with the GPhC
Training and assessment	• Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. • All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines • All users must previously have used PGD's to supply medication • Must work in compliance with the SOPs of their own employer • All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	• All users must be up to date with CPD requirements and appraisal • All users must be up to date with MHRA and safety alerts with regards to medical products • All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

PGD Indication	Active immunisation against varicella (chickenpox) in individuals aged 12 months and over who are susceptible to the varicella-zoster virus and have no history of chickenpox infection or prior vaccination.
Inclusion criteria	- Individuals aged 12 months and older. - No history of chickenpox infection. - No previous varicella vaccination. - Informed consent given. - Healthy susceptible individuals who may be exposed to varicella virus in the course of their work - Healthy susceptible healthcare workers who are in regular or close contact with immunocompromised individuals to prevent transmission - Healthy susceptible close household contacts of immunocompromised patients where close contact is unavoidable - Healthy susceptible individuals who have been exposed to varicella virus within 3 days of exposure
Exclusion criteria	- History of chickenpox infection. - Previous varicella vaccination. - Hypersensitivity to neomycin, gelatin, or any component of the vaccine. - Immunocompromised individuals unless under specialist advice. - Pregnancy or planning pregnancy within one month. - Breastfeeding - Acute febrile illness or active untreated tuberculosis. - Individuals receiving salicylate therapy (other than topical treatment for localised conditions) because of the association of Reye's syndrome with salicylates.
Cautions	- Avoid contact with high-risk individuals (e.g. immunosuppressed) for 4–6 weeks if a rash develops post-vaccination. - Pregnancy must be avoided for one month post-vaccination. - Postpone in the case of moderate or severe illness with fever. - Consider delaying vaccination if the individual received blood products recently. - Vaccine recipients should avoid the use of salicylates for at least 6 weeks after vaccination - Aciclovir has been shown to reduce the activity of the vaccine. To ensure adequate vaccine response, avoid vaccination until at least 24 hours after completion of aciclovir therapy. After being vaccinated it is best to wait at least 14 days before using aciclovir. - Caution is advised for elderly patients (≥ 65 years of age) as there is no data on the immune responses in these individuals. - Varicella vaccines contain live strains of the varicella virus. Where a patient has received a live vaccine in the last 4 weeks or another live vaccine is planned within 4 weeks of the administration of a chicken pox vaccine, Green Book recommendations should be followed:

	"Either varicella vaccine product can be given at the same time as other vaccines such as MMR, DTaP/ IPV, Hib/MenC, PCV, hepatitis B and Men B. If varicella vaccine cannot be given at the same time as an inactivated vaccine, it can be given at any interval before or after. If varicella vaccine cannot be administered on the same day as MMR, then a 4-week minimum interval should be observed between the vaccines. For all other live vaccines e.g LAIV no deferral is required." In individuals who have received immunoglobulins or a blood transfusion, vaccination should be delayed for five months for Varivax® because of the likelihood of vaccine failure due to passively acquired antibody to the VZ virus. Caution should be used in individuals with generalised septic skin conditions as the rash produced may be more severe. If eczema exists, a site free from skin lesions must be chosen If tuberculin testing (Mantoux test) has to be done it should be carried out before or simultaneously with vaccination since it has been reported that live vaccines may cause a temporary depression of tuberculin sensitivity
Actions if patient is excluded or declines treatment	Advise on alternative treatment options and how these can be accessed. Document any advice given and the decision reached. Inform or refer to the GP as appropriate

Details of the medicine

Name, form and strength of medicine	Varivax powder and solvent for suspension for injection 0.5mls
Legal category	POM
Storage	Store in a refrigerator (2°C–8°C). Do not freeze. Protect from light.
Route/method of administration	The vaccine is to be injected intramuscularly (IM) or subcutaneously (SC). The preferred injection sites are the anterolateral area of the thigh in younger children and the deltoid area in older children, adolescents, and adults. The vaccine should be administered subcutaneously in patients with thrombocytopenia or any coagulation disorder. When administering at the same time as other vaccines, they should be given at separate sites, preferably in different limbs. If given in the same limb, they should be given at least 2.5cm apart.
Dose and frequency	Two doses are recommended for optimal protection. For individuals from 12 months to 12 years of age, at least one month must elapse between the first and second dose. Individuals from 13 years of age and older should receive two doses given 4-8 weeks apart. If the interval between doses exceeds 8 weeks, the second dose should be given as soon as possible.

	NB: A single dose of Varivax may be administered to those who have already received a single dose of another varicella-containing vaccine. A single dose of Varivax may be administered followed by a single dose of another varicella-containing vaccine.
Quantity to be administered and/or supplied	Single 0.5ml dose
Maximum or minimum treatment period	Two doses are recommended for optimal protection Based on clinical discretion, varicella vaccination may be considered for seronegative individuals who are planning to receive immunosuppressive treatment. If there is sufficient time to complete the two-dose course, this should be given. If treatment must begin sooner, they should be offered a single dose.
Adverse effects	Injection site reactions (pain, redness, swelling), fever, rash, irritability, or upper respiratory symptoms. Rarely, varicella-like rash or febrile convulsions. Refer to SmPC. Adrenaline injection 1mg/ml (1:1000) must always be available whenever vaccines are given. Immediate treatment for anaphylaxis should include early administration of intramuscular adrenaline and a call to the emergency services. The healthcare professional delivering the service must be trained to recognise an anaphylactic reaction and be familiar with techniques for resuscitation of a patient with anaphylaxis. Fainting can occur following, or even before, any vaccination especially in adolescents. It is important that procedures are in place to avoid injury from faints.
Yellow card reporting	Report any suspected adverse effects via https://yellowcard.mhra.gov.uk and notify the GP as appropriate.

Records to be kept	Record the following information: ● that valid informed consent was given ● name of individual, address, date of birth and GP ● name of healthcare practitioner ● name and brand of medication ● date of supply dose, form and route quantity supplied ● advice given, including advice given if excluded or declines treatment ● details of any adverse drug reactions and actions taken supplied via PGD Records should be signed and dated. All records should be clear, legible and contemporaneous. Electronic records can be made. A record of all individuals receiving treatment under this PGD should also be kept for audit purposes as follows: ● Adults aged 18 years and over - keep records for 8 years ● Children aged under 18 years -keep records until the 25th birthday (if the patient was 17 years when treatment was finished, keep records until the 26th birthday)
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Patient information

Written information to be given to patient or carer	Supply the patient information leaflet (PIL) provided with the medication.
Follow-up advice to be given to patient or carer	To seek medical advice if symptoms worsen rapidly or significantly, do not improve in 3–4 weeks, or they become systemically very unwell. Patients should be warned that no vaccine is 100% effective. Inform patients that maximum protection will occur approximately 6 weeks after the second dose. Advise that most adverse reactions disappear within 1-2 days. If the symptoms do not disappear, or the patient experiences confusion or persistent drowsiness then they should be advised to seek medical advice Patients whose contact with high-risk individuals is unavoidable should be advised to carefully monitor themselves for rashes from the first dose until at least 6 weeks after the second dose. They should seek medical advice if a post-vaccine rash appears.

Key references

- NICE Medicines Practice Guideline 2 (MPG2): Patient Group Directions. Published March 2017 <https://www.nice.org.uk/guidance/mpg2>
- NICE MPG2 Patient group directions: competency framework for health professionals using patient group directions. Updated March 2017 <https://www.nice.org.uk/guidance/mpg2/resources>
- NICE CKS Guidance
- British National Formulary (BNF) Key information on the selection, prescribing, dispensing and administration of medicines [BNF \(British National Formulary\) | NICE](#)
- Summary of Product Characteristics for drug

Agreement to practise by the Registered Healthcare Professional

By signing this PGD you are confirming that you agree to its contents and that you will work within it. PGDs do not remove inherent professional obligations or accountability. It is the

responsibility of each healthcare professional to practise only within the bounds of their own competence and professional code of conduct.

Name and address of pharmacy premises / clinic premises to which this PGD relates			
Name of healthcare professional	Job title	Registration number	Signature
Valid from date		Expiry date	
1/2/26		30/9/26	

PGD adoption and authorisation by the employer/clinical lead

The employer has ensured that the person using this PGD, has the knowledge and skills to safely provide the service which will encompass the supply or administration of a medication or vaccine.

This section must be signed by the superintendent or clinical lead responsible for this service.

Name of superintendent / clinical lead	Job title & name of organisation	Signature	Date
Name of professional group signatory	Job title & name of organisation	Signature	Date

Signed on behalf of Get Real Health

Name	Qualification	Signature	Date
Nitin Shori	Medical Doctor GMC: 6047293		1/2/26
Chris Pilkington	Pharmacist GPhC: 2046322		1/2/26

Change history

Version number	Change details	Date
001	Development & issue of new PGD	1 st Feb 2026

This Patient Group Direction (PGD) is intended only for registered healthcare professionals who have been named and authorised by their organisation to practice under it.

Patient Group Direction
for the administration of Varilrix for the treatment of Chickenpox

Patient Group Direction

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Healthcare professionals covered and training

	Healthcare professionals allowed to work under this PGD and requirements
Qualifications and Registration	● Pharmacist registered and practicing with the GPhC ● Pharmacy technician registered and practicing with the GPhC
Training and assessment	● Pharmacists and Pharmacy Technicians must have completed training relevant to this condition to use this PGD. This training will be documented and overseen by the Get Real Health team. ● All users must be familiar with the SPC for the products as well as BNF advice and any applicable national guidelines ● All users must previously have used PGD's to supply medication ● Must work in compliance with the SOPs of their own employer ● All users must have appropriate indemnity in place to cover this service
Further training and responsibilities	● All users must be up to date with CPD requirements and appraisal ● All users must be up to date with MHRA and safety alerts with regards to medical products ● All users must understand the concepts of capacity and consent and be aware of the Mental Capacity Act 2005.

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Exclusion criteria	- History of chickenpox infection. - Previous varicella vaccination. - Hypersensitivity to neomycin, gelatin, or any component of the vaccine. - Immunocompromised individuals unless under specialist advice. - Pregnancy or planning pregnancy within one month. - Breastfeeding - Acute febrile illness or active untreated tuberculosis. - Individuals receiving salicylate therapy (other than topical treatment for localised conditions) because of the association of Reye's syndrome with salicylates.
Cautions	- Avoid contact with high-risk individuals (e.g. immunosuppressed) for 4–6 weeks if a rash develops post-vaccination. - Pregnancy must be avoided for one month post-vaccination. - Postpone in the case of moderate or severe illness with fever. - Vaccine recipients should avoid the use of salicylates for at least 6 weeks after vaccination - Aciclovir has been shown to reduce the activity of the vaccine. To ensure adequate vaccine response, avoid vaccination until at least 24 hours after completion of aciclovir therapy. After being vaccinated it is best to wait at least 14 days before using aciclovir. - Caution is advised for elderly patients (≥65 years of age) as there is no data on the immune responses in these individuals. - Varicella vaccines contain live strains of the varicella virus. Where a patient has received a live vaccine in the last 4 weeks or another live vaccine is planned within 4 weeks of the administration of a chicken pox vaccine, Green Book recommendations should be followed: "Either varicella vaccine product can be given at the same time as other vaccines such as MMR, DTaP/ IPV, Hib/MenC, PCV, hepatitis B and Men

	B. If varicella vaccine cannot be given at the same time as an inactivated vaccine, it can be given at any interval before or after. If varicella vaccine cannot be administered on the same day as MMR, then a 4-week minimum interval should be observed between the vaccines. For all other live vaccines e.g LAIV no deferral is required." In individuals who have received immunoglobulins or a blood transfusion, vaccination should be delayed for at least three months for Varilrix® because of the likelihood of vaccine failure due to passively acquired antibody to the VZ virus. Caution should be used in individuals with generalised septic skin conditions as the rash produced may be more severe. If eczema exists, a site free from skin lesions must be chosen If tuberculin testing (Mantoux test) has to be done it should be carried out before or simultaneously with vaccination since it has been reported that live vaccines may cause a temporary depression of tuberculin sensitivity
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Details of the medicine

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Legal category	POM
Storage	Store in a refrigerator (2°C–8°C). Do not freeze. Protect from light.
Route/method of administration	Varilrix is to be injected subcutaneously (SC) or intramuscularly (IM) in the deltoid region or in the anterolateral area of the thigh. Varilrix should be administered subcutaneously in individuals with bleeding disorders (e.g. thrombocytopenia or any coagulation disorder). When administering at the same time as other vaccines, they should be given at separate sites, preferably in different limbs. If given in the same limb, they should be given at least 2.5cm apart.
Dose and frequency	Children from the age of 12 months as well as adolescents and adults receive two doses of Varilrix to ensure optimal protection against varicella. The second dose should generally be given at least 6 weeks after the first dose. Under no circumstances should the interval between the doses be less than 4 weeks. NB: A single dose of Varilrix may be administered to those who have already received a single dose of another varicella-containing vaccine. A single dose of Varilrix may be administered followed by a single dose of another varicella-containing vaccine.

Quantity to be administered and/or supplied	0.5ml dose
Maximum or minimum treatment period	Two doses are recommended for optimal protection. Based on clinical discretion, varicella vaccination may be considered for seronegative individuals who are planning to receive immunosuppressive treatment. If there is sufficient time to complete the two-dose course, this should be given. If treatment must begin sooner, they should be offered a single dose.
Adverse effects	Injection site reactions, fever, rash, irritability. Rare: febrile seizures, varicella-like rash. Refer to SmPC for full adverse effect profile. Adrenaline injection 1mg/ml (1:1000) must always be available whenever vaccines are given. Immediate treatment for anaphylaxis should include early administration of intramuscular adrenaline and a call to the emergency services. The healthcare professional delivering the service must be trained to recognise an anaphylactic reaction and be familiar with techniques for resuscitation of a patient with anaphylaxis. Fainting can occur following, or even before, any vaccination especially in adolescents. It is important that procedures are in place to avoid injury from faints.

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